

1. Administrative & Study Identification

Full Study Title: A Retrospective Observational Study on the Long-term Effects of Ipilimumab-treated Pediatric Participants in the Dutch Melanoma Treatment Registry (DMTR)
Short Title/Acronym: DMTR Pediatric Ipilimumab Observational Study
Protocol Number: CA184-557
NCT Number: NCT04196452
Sponsor/Company Name: Bristol Myers Squibb
Investigational Product: Ipilimumab (Trade Name: Yervoy; ATC Code: L01FX04)
Phase of Development: Not Applicable
Medical Monitor: BMS Clinical Trials Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective & Summary * **Summary:** This is an observational, national, retrospective study consisting of pediatric patients with advanced (spread or unremoveable) melanoma identified in the DMTR in the Netherlands. * **Primary Objective:** To retrospectively evaluate the long-term safety, clinical effects, and tolerability profile of ipilimumab treatment in pediatric participants (0-18 years) with advanced melanoma using the Dutch Melanoma Treatment Registry (DMTR). * **Secondary Objectives:** To monitor for late-onset immune-related adverse events and to gather long-term real-world outcomes to supplement pivotal pediatric clinical trials.

2.2 Background & Rationale While immune checkpoint inhibitors have transformed the treatment of advanced melanoma in adults, pediatric specific safety and long-term efficacy data remain limited due to the rarity of the disease in children. This post-authorization observational study utilizes real-world data from the DMTR to track long-term effects, addressing the regulatory need to extrapolate and verify the safety of ipilimumab treatment in the adolescent and pediatric population.

3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Observational (Retrospective Cohort) * **Control Type:** Single-arm (Registry-based) * **Blinding:** Open-label / Non-blinded * **Randomization:** N/A (Observational data extraction)

3.2 Planned Enrollment & Duration * **Target** \$N\$: 10 * **Number of Sites**: Single registry source (Dutch Melanoma Treatment Registry, Netherlands) * **Estimated Study Start**: Retrospective data collection * **Trial Status**: ACTIVE_NOT_RECRUITING (Active but not currently recruiting)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Less than 18 years of age at first dose of monotherapy Ipilimumab used for treatment of advanced melanoma. 2. Histological or cytological confirmation of advanced (unresectable or metastatic) melanoma.

4.2 Exclusion Criteria 1. Participation in a clinical trial within the past 4 weeks prior to first dose with ipilimumab or concurrently. 2. *(Other inclusion/exclusion criteria apply as per DMTR protocols)*

5. Intervention & Treatment Plan

5.1 Investigational Regimen * **Dosage/Frequency**: Standard clinical practice dosing of Ipilimumab documented retrospectively in the DMTR. * **Route of Administration**: Intravenous (IV) * **Duration of Treatment**: Based on real-world clinical practice and physician discretion; patients are followed longitudinally.

5.2 Concomitant & Prohibited Therapy * **Permitted**: Standard supportive care and concomitant therapies for advanced melanoma as recorded in the registry. * **Prohibited**: Concurrent enrollment in another investigational trial within 4 weeks prior to the initial ipilimumab dose.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures		
:---	:---	Data Extraction	Retrospective	Extraction of baseline demographics, disease characteristics, and historical treatment protocols from DMTR.
		Long-term Follow-up	Continuous	Periodic registry updates for survival outcomes and documentation of late-onset immune-related adverse events.

(Note: As a retrospective registry study, traditional prospective patient visit schedules do not apply.)

7. Outcome Measures (Endpoints)

7.1 Primary Outcomes 1. Frequency of Adverse Events (AEs) Grades 3-4.

7.2 Secondary Outcomes 1. Baseline assessment of population demographics. 2. Baseline assessment of comorbidities. 3. Baseline assessment of disease characteristics.

8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Extracted retrospectively from the DMTR; special focus on Frequency of AEs Grades 3-4.
 - **Serious Adverse Events (SAEs):** Identification of severe toxicities or hospitalization events linked to prior ipilimumab treatment.
 - **Data Monitoring Committee (DMC):** Governed by DMTR data protocols and BMS internal pharmacovigilance safety teams.
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9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** All pediatric patients (target N=10) in the DMTR with confirmed advanced melanoma who received ipilimumab.
 - **Handling of Missing Data:** Standard epidemiological imputation or listwise deletion as appropriate for retrospective real-world data analysis.
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10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** Conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and local data privacy regulations.
 - **Monitoring Plan:** Verification of extracted variables against source records stored within the Dutch Melanoma Treatment Registry.
 - **Product Information:** [EMA Yervoy EPAR Product Information](#)
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11. Study Identifiers & Governance

- **Posting ID:** 326035254184590071
 - **Primary ID:** CA184-557
 - **Registry Number:** NCT04196452
 - **Sponsor/Lead Organization:** Bristol Myers Squibb
 - **Study Title:** A Retrospective Observational Study on the Long-term Effects of Ipilimumab-treated Pediatric Participants in the Dutch Melanoma Treatment Registry (DMTR)
 - **Official Regulatory Title:** Long-term Follow-up of Ipilimumab-treated Pediatric Patients Enrolled in the Dutch Melanoma Treatment Registry (DMTR)
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12. Clinical Framework (Melanoma Specific)

- **Disease Areas:** Melanoma (Preferred UMLS Name: Pediatric Melanoma)
 - **Medical Classification Codes:**
 - MeSH Code: D008545
 - HPO Code: HP:0002861
 - SNOMED ID: 372244006
 - **Condition Definition:** A malignant neoplasm derived from cells that are capable of forming melanin, occurring in skin, eyes, or mucous membranes. It frequently metastasizes widely.
 - **Medical Methodology:** Immune Checkpoint Inhibition (Standard of Care)
 - **Optimization Target:** Long-term disease control, survival, and management of immune-related toxicities
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13. Study Procedures & Methodology

- **Management Pathway:** Registry-based retrospective data extraction
 - **Education Protocol (HCP):** N/A – Retrospective observational study
 - **Education Protocol (Patient):** N/A – Retrospective observational study
 - **Quality Audit Mechanism:** DMTR data verification and registry governance audits
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14. Observation & Follow-Up Schedule

- **Total Duration:** Long-term longitudinal follow-up
 - **Onsite Visit Cadence:** N/A – Retrospective registry study
 - **Remote Monitoring Frequency:** Periodic continuous data extraction from DMTR
 - **Checklist Review Interval:** Routine aggregate safety data and pharmacovigilance reviews
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15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				